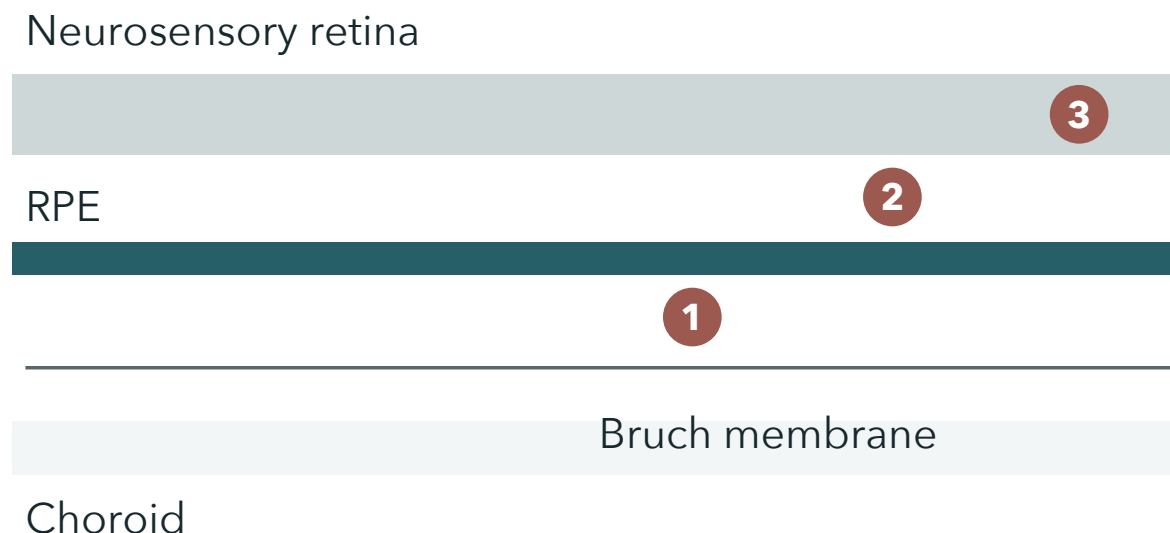


Macular neovascular phenotypes and treatment evidence

Part 2 | Map the lesion before choosing a regimen

Structural location and exudative activity answer different questions.



Original layer sketch: 1 below RPE, 2 above RPE, 3 intraretinal.

A vascular network establishes a phenotype; new fluid, haemorrhage and functional change indicate activity. A tall or notched pigment epithelial detachment and recurrent haemorrhage should prompt reconsideration of PCV rather than an automatic label of treatment resistance. Interpret scans alongside the fundus and angiography.

EVEREST II: what was actually compared

Prompt PDT with ranibizumab improved the studied outcomes.

Finding	Implication
Population	ICGA-confirmed symptomatic PCV
Comparison	Ranibizumab with prompt PDT versus ranibizumab alone
Two-year outcome	Better vision gain and more complete polyp regression
Treatment burden	Fewer ranibizumab injections with combination

This is evidence for the specified combination, not proof that PDT must accompany every anti-VEGF agent. Ask whether the patient, drug, timing and retreatment approach match the trial before translating its result into a clinical rule.

PLANET: understand selective rescue

Rescue designs cannot establish every benefit of prompt combination therapy.

Finding	Implication
Agent	Aflibercept in both groups
PDT strategy	Rescue treatment for a suboptimal response
Main finding	Most eyes responded without requiring rescue
Inference limit	Prompt combination was not tested in every eye

Good outcomes with aflibercept monotherapy support that option. Few participants needed rescue PDT, limiting what can be concluded about its added benefit. Do not read a noninferiority result as evidence that PDT is never useful.

Select and counsel about PDT

Photochemical treatment has its own safety considerations.

- Map the lesion using appropriate imaging
↓
- Agree the drug and PDT strategy
↓
- Check verteporfin suitability and precautions
↓
- Monitor function and anatomical response

Half-dose and half-fluence are not interchangeable descriptions. The appropriate protocol depends on the disease and supporting evidence. Explain the intended target, expected benefit, alternatives and post-treatment precautions, using current product information rather than recalling an unqualified historical schedule.

Poor recovery is not always ongoing leakage

Identify what remains reversible before escalating.

Finding	Implication
Fluid or new haemorrhage	Review activity and regimen adequacy
Fibrosis or atrophy	Structural limit to visual recovery
Persistent abnormal PED	Revisit phenotype and relevant imaging
Good anatomy but poor vision	Review other retinal and optical causes

When reviewing a difficult case, state the remaining mechanism before proposing another treatment. The useful question is whether loss reflects active exudation, irreversible damage, an incompletely characterised lesion or another cause. Each interpretation leads to a different benefit-risk discussion.